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Name: ايمان محمد شافعي محمد National ID: 27606130102023	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient Name : الأستاذة / ايمان محمد شافعي محمد	Request No. : 13003558
Patient No. : 27606130102023	Request Date : 09/08/2022
Path No. : 2260/22	Verification Date : 29/08/2022
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 46 Y	Department : Out Patient
Referreing Doc. :	IHC No. :
	Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Left UOQ lesion and intramammary lymph node at left axillary tail.

Nature of the specimen: Tru cut biopsy collected at 8/8/2022 at 11.58pm.

Gross Examination: Two biopsies were recived:

- 1- Left breast : Multiple grayish white tissue cores up to 1.5cm.
- 2- Left axillary lymph node: Multiple grayish white tissue cores up to 1cm.
- Each was totally submitted.

Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5)

Microscopic examination:

1- Left breast : reveals diffuse stromal fibrosis and focal adenosis. Some of the proliferating ducts are irregularly dilated. The ducts are lined by inner columnar and outer myoepithelial cells. There is no evidence of malignancy in sections examined.

2- Left axillary lymph node: Metastatic poorly differntiated carcinoma .

-IPT:

GATA3:Positive

ER: No nuclear reaction in tumor cells (Allred score 0/8)

PR: No nuclear reaction in tumor cells (Allred score 0/8)

Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)

Diagnosis:

Left UOQ lesion and intramammary lymph node; Tru cut biopsy:

1- Left breast : Fibroadenosis.

2- Left axillary lymph node: Metastatic poorly differntiated carcinoma.

- IPT

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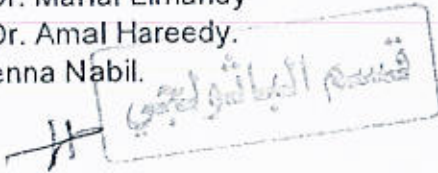


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Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

GATA3:Positive
- ER : Negative (0/8)
- PR : Negative (0/8)
- HER2 : Positive(score 3+)

Prof. Dr. Manal Elmahdy
Prof. Dr. Amal Hareedy.
Dr. Menna Nabil.



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MUHAMMED SHAFEAY, EMAN Patient ID: 122021676 DOB: 13-May-1987 Date: 12-Apr-2025

Patient Lastname: Muhammed Shafeay

Patient Firstname: Eman

HIS Patient ID: 122021676

RIS Patient ID: 312367

Date of Birth: 13/05/1987

Procedure: 69269, CT Chest Abdomen Pelvis with Contrast

Medical Question:

Clinical Comment: عيادة الجراحة

===== REPORT TEXT =====

EXAMINATION: CT Scan of the Chest, Abdomen and Pelvis With Contrast.

TECHNIQUE: Contiguous helical axial images of the chest, abdomen and pelvis were obtained from the base of the neck down to the symphysis pubis after administration of intravenous contrast.

CLINICAL HISTORY:

COMPARISON:

FINDINGS:

1. Chest: compared to 16/2/23

" stationary previously seen small calcific pleural plaques more on the right side otherwise The lungs are clear bilaterally without evidence of consolidation, pulmonary masses or nodules. No pulmonary parenchymal disease is noted.

" No mediastinal or hilar lymphadenopathy.

" Normal cardiac size.

" Unremarkable appearance of the thoracic aorta.

" No pericardial or pleural effusion.

2. Abdomen and Pelvis:

" enlarged fatty liver 22.5 cm with no sizable focal lesions

" The spleen, the pancreas, both adrenal glands and left kidney are within normal limits.

" no sizable operative bed masses

" few small peritoneal nodules/ lymph nodes are seen largest 1 cm likely reactive lymph nodes for follow up

" The abdominal aorta is of normal diameter. There is no evidence of abdominal aortic aneurysm.

" The visualized portion of the bowel is unremarkable.

" No ascites.

" The pelvic organs are within normal limits.

" The visualized portions of the skeleton are unremarkable.

IMPRESSION:

" no CT evidence of recurrent operative bed disease

" few peritoneal nodules likely reactive lymph nodes for follow up

dr Manal Refaat

===== REPORT TEXT ENDE =====



Patient Name: Eman Mohamed Shafie Mohamed	Date : 08/04/2025
Exam : C.T CHEST	Age : 48 Year(s)
Medical ID: 2289/2022	Pat. ID: 13435

- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus with regressive course of the right adnexal marginally thick walled cystic lesion measuring 3 cm.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No osseous lytic or sclerotic lesions.
- Dorsolumbar spondylosis.

Opinion:

Status follow up showing:

- Regressive course of the right adnexal cystic lesion.
- Otherwise, overall stationary course since last study.

Dr. Mohamed Mohsen, M.D.